



ROOT CAUSE UNLOCKED · CONDITION KIT

The Fibromyalgia *Supplement* Guide

I use supplements in practice and I think they earn their place, but they earn it as the assist rather than the play, which is why this guide grades every ingredient on that basis and names the ones that are prescriber conversations rather than shopping-cart decisions.

Dr. Adam Seay, DC

Root Cause Unlocked, Garner, NC · rootcauseunlocked.com · (919) 662-0520

The Fibromyalgia Supplement Guide

BEFORE YOU START

Please read this first

This guide is educational. It is not medical advice, it does not diagnose anything, and it does not replace your own clinician. Nothing here is a treatment for fibromyalgia, and nothing here is a substitute for the care you are already receiving.

Talk with your prescriber before you start, stop, or change any supplement. *Fibromyalgia is commonly managed with medications (duloxetine, milnacipran, amitriptyline, pregabalin, gabapentin, cyclobenzaprine, tramadol, SSRIs) that interact with several of the ingredients discussed in these pages. Some of those interactions are serious. The safety sections are not filler. They are the most important part of this guide.*

If you are pregnant, nursing, trying to conceive, under 18, have kidney or liver disease, a bleeding disorder, or take a blood thinner, treat every entry here as “ask first” rather than “try it.”

Supplements in the United States are regulated as food, not as drugs. Quality varies. Choose products that carry independent third-party testing, and bring the actual label to your appointments.

Contents

What you are actually buying back

Supplements are supportive, not magic

How to read this guide

Before you start: the medication conversation

Vitamin D

Magnesium

Omega-3 fatty acids (EPA and DHA)

If sleep is the problem

If pain is the problem

If fatigue and post-exertion crashes are the problem

If your gut is part of the picture

5-HTP (5-hydroxytryptophan)

SAMe (S-adenosylmethionine)

St John's wort

Iron, only if your labs say so

What I deliberately left out, and why

Running a fair 12-week trial

Realistic expectations

When to seek professional care

Where to find quality versions of these

Working with Root Health

References

What you are actually buying back

Nobody wakes up wanting a supplement routine. What people with fibromyalgia want back is specific: a night of sleep that actually restores something. A morning that does not start at a four out of ten before your feet hit the floor. A week with fewer flare days than good days. Enough steady energy to finish what you started.

That is the frame for this entire guide. Every entry gets judged against one question: does it help you get a piece of your life back? If the honest answer for a given ingredient is “the evidence is thin,” you will read that here in plain language instead of a sales pitch.

Supplements are supportive, not magic

Here is the part most supplement guides skip.

Fibromyalgia involves the way your nervous system processes pain signals, and it usually travels with three companions: unrefreshing sleep, fatigue, and gut symptoms. No capsule reprograms a sensitized nervous system. The interventions with the strongest evidence base in fibromyalgia are not supplements at all. European rheumatology guidelines put graded exercise at the top of the list, with education, sleep work, and psychological and physical therapies close behind, and they rate most other options far lower (PMID 27377815).

I use supplements in practice, and I think they earn their place. But they earn it as support: correcting a real nutrient shortfall, taking pressure off sleep, giving mitochondria better raw material, calming a gut that is feeding into the whole picture. They are the assist, not the play.

So the honest sequence is:

1. **Movement you can actually sustain.** Even small amounts, built slowly, done consistently.
2. **Sleep as a project**, not an afterthought.
3. **Food that steadies you** (see the companion Fibromyalgia Diet Guide).
4. **Then targeted supplements**, ideally guided by actual lab values.
5. **Alongside whatever medical care you are already receiving** (PMID 38927473).

If you skip the first three and buy the fourth, you will spend money and feel unchanged. I would rather tell you that now than let you learn it over six months.

How to read this guide

The tiers. Foundational means “most likely to matter for most people with fibromyalgia, and easiest to get right.” Targeted means “pick based on which symptom is running your life.” Ask-first means “the interaction risk is real enough that this belongs in a conversation with your prescriber before you order anything.”

Typical studied range. Where a dose appears, it is the range used in the published research being cited, presented so you can discuss it intelligently with your clinician. It is not a prescription for you. Your kidneys, your liver, your medications, your labs, and your body weight all change the right answer.

The claims language is deliberate. You will read “supports,” “may help,” and “is being studied for.” You will not read “treats,” “fixes,” or “reverses,” because those words would be false, and because they are how people end up spending hundreds of dollars on hope.

One change at a time. Start one product, hold everything else steady, give it six to eight weeks, and write down what happened. Starting five things at once guarantees you will never know which one earned its place, and it makes any side effect impossible to trace.

Track something simple. Two numbers, most days: pain out of ten, and sleep quality out of ten. Add a flare-day tally per week. Twelve weeks of that beats any amount of “I think it might be helping.”

Why the evidence for nutrients looks thinner than it is. A large outcome trial costs tens to hundreds of millions of dollars, and that money only exists when a patent can pay it back. Nobody can patent magnesium, so the hierarchy this guide grades against systematically over-documents patentable drugs and under-documents nearly everything else. A Cochrane methodology review quantified the tilt: manufacturer sponsorship produces more favourable results and conclusions than independent funding, through a bias that standard quality checks do not catch (PMID 28207928). So when this guide says “no evidence,” it will tell you which of three things it means: *tested and failed* (a real trial ran and the answer was no), *never properly tested* (nobody could afford the trial, so absence of evidence is not evidence of absence), or *tested small and won* (real but modest trials, usually on markers rather than outcomes, because markers are what a small budget can afford).

The same skepticism runs in both directions. The asymmetry earns nutrients humility, not automatic credit: when isolated nutrients have received large public trials the results were mixed, and small nutrient trials carry their own publication bias toward positive findings, just as drug trials tilt toward their sponsor. This guide names the funding and the evidence tier on both sides and lets you see them.

Before you start: the medication conversation

Fibromyalgia care very often includes serotonin-active medication. That single fact drives most of the safety content in this guide.

Serotonin-active prescriptions commonly used in fibromyalgia: duloxetine, milnacipran, venlafaxine, amitriptyline and other tricyclics, SSRIs (fluoxetine, sertraline, escitalopram, paroxetine, citalopram), tramadol, and triptans for migraine. Cyclobenzaprine is structurally related to tricyclics and belongs on this list too.

Supplements that add serotonin activity on top of those: 5-HTP, SAMe, St John's wort, L-tryptophan, and some combination "mood" or "sleep" blends that contain them without saying so on the front of the label.

Combining serotonin-active drugs with serotonin-active supplements can cause serotonin syndrome, which ranges from agitation, sweating, tremor, and diarrhea to a medical emergency with high fever and muscle rigidity (PMID 37309284). This is the single most important safety point in this document. If you take any medication on that first list, the ingredients on the second list are prescriber conversations, not shopping-cart decisions.

Other interaction categories worth naming:

- **Sedating medications** (pregabalin, gabapentin, cyclobenzaprine, benzodiazepines, sleep aids, opioids) stack with sedating supplements such as melatonin. More grogginess, more fall risk.
- **Blood thinners and antiplatelet drugs** (warfarin, apixaban, clopidogrel, daily aspirin) stack with higher-dose fish oil and with several botanicals. Bleeding risk is the concern.
- **St John's wort** speeds up the liver's clearance of a long list of drugs, including oral contraceptives, some blood thinners, transplant medications, and several antidepressants, and can cause serious loss of effect (PMID 31742659). Do not self-start it.
- **Kidney or liver disease** changes the safety of magnesium, creatine, and fat-soluble vitamins.
- **Pregnancy and nursing:** almost nothing in this guide has been studied for safety in pregnancy. Default to "not without your obstetric provider's sign-off."

Bring the label. Not the marketing name, the actual ingredient panel. Half of the interaction problems I see come from blends where the risky ingredient is fourth on the list.

Tier 1: The foundational few

These are the ones with either the most consistent fibromyalgia-specific research or the most plausible chance of correcting something genuinely low. Start here, and ideally start with labs.

Vitamin D

What it is. A fat-soluble vitamin your skin makes from sunlight and your liver and kidneys activate. Low levels are common in people with chronic pain and in people who spend most of their time indoors, which describes a lot of fibromyalgia life.

Why it may help. Vitamin D deficiency shows up frequently in fibromyalgia populations, and reviews have looked at whether it tracks with symptom severity (PMID 35071984). In a small randomized placebo-controlled trial of women with fibromyalgia and low vitamin D, bringing blood levels up over 20 weeks was associated with a meaningful reduction in pain scores compared with placebo (PMID 24438771). A 2022 meta-analysis of five randomized studies covering 315 participants found that vitamin D supplementation improved Fibromyalgia Impact Questionnaire scores, though it did not find a statistically significant difference on the pain visual analogue scale (PMID 35596576). Read that split carefully: overall symptom impact improved, the pain-only measure did not reach significance, and the studies were small.

Typical studied range. The trials were designed to move a low blood level into the normal range rather than to test one fixed dose, which is exactly why the blood test comes first (PMID 24438771). Ask your clinician to check 25-hydroxy vitamin D and to set the dose from the result. Retest rather than assuming.

Timing. With a meal containing fat, which improves absorption.

Cautions. Vitamin D is fat-soluble and accumulates. High doses over time can raise blood calcium, which is not benign. It interacts with thiazide diuretics, some seizure medications, and digoxin. This is a test-and-dose nutrient, not a “more must be better” nutrient.

Magnesium

What it is. A mineral involved in hundreds of enzyme reactions, including muscle relaxation, nerve signaling, and energy production. Common forms are glycinate (gentler on the gut, often chosen for evening use), citrate (more likely to loosen stool), malate, and oxide (poorly absorbed, mostly a laxative).

Why it may help. A controlled study in 60 premenopausal women with fibromyalgia found that blood and red-cell magnesium levels were lower than in healthy controls, and that eight weeks of magnesium citrate at 300 mg daily reduced tender point counts, tender point index, Fibromyalgia Impact Questionnaire scores, and depression scores, with broader improvement when it was combined with low-dose amitriptyline (PMID 22271372). A literature review of magnesium in fibromyalgia concluded that the rationale is reasonable and the human evidence is still limited (PMID 34392734). A separate evidence summary looking specifically at magnesium plus malic acid found the certainty of evidence low and the effect uncertain (PMID 31150373). So: worth trying, honestly described as promising rather than proven.

Typical studied range. 300 mg daily of elemental magnesium as magnesium citrate for eight weeks in the trial above (PMID 22271372).

Timing. Evening tends to suit people best, especially glycinate, because of the muscle and sleep angle.

Cautions. Loose stools are the usual limiting side effect, and citrate is the worst offender for that. Magnesium can interfere with the absorption of some antibiotics, thyroid medication, and bisphosphonates, so separate them by several hours. **If you have reduced kidney function, do not take magnesium without your clinician's approval.** Impaired kidneys cannot clear it well.

Omega-3 fatty acids (EPA and DHA)

What it is. The long-chain fats found in oily fish, with algae-derived versions available for people who do not eat fish.

Why it may help. In a randomized, double-blind, placebo-controlled trial of 120 patients with fibromyalgia, eight weeks of a high oral dose of omega-3 was associated with significant improvement in the Widespread Pain Index, Symptom Severity Scale, pain visual analogue scale, and Fibromyalgia Impact Questionnaire compared with placebo (PMID 39377412). That is a single trial, and it deserves confirmation, but it is a reasonably sized one with patient-reported outcomes that matter. Broader nutrition reviews of chronic pain also place omega-3 intake inside an anti-inflammatory dietary pattern rather than treating it as a standalone remedy (PMID 29679994).

Typical studied range. The fibromyalgia trial tested a high dose over eight weeks and did not compare lower doses (PMID 39377412). Across pain and inflammation research generally, the range studied most often is 1 to 3 grams per day of combined EPA plus DHA. Note that this refers to actual EPA plus DHA content, not total “fish oil” on the front of the bottle, which is usually a much bigger number.

Timing. With meals. Splitting the dose reduces fishy repeat.

Cautions. Higher doses can increase bleeding tendency, which matters if you take a blood thinner, an antiplatelet drug, or daily aspirin, or if you have surgery scheduled. Ask your surgeon about stopping in advance. Fish allergy is an obvious exclusion, and algae-based products are the workaround.

Tier 2: Targeted support

Pick by the symptom that is costing you the most, not by the length of the list.

If sleep is the problem

Unrefreshing sleep is not a side issue in fibromyalgia. It sits close to the center, and it feeds pain the next day. Anything that genuinely improves sleep tends to pay you back twice.

Melatonin

What it is. A hormone your brain releases as darkness falls. It is a timing signal more than a sedative, which is why the timing of the dose matters as much as the amount.

Why it may help. In a double-blind, placebo-controlled study of 101 patients with fibromyalgia, melatonin at 3 mg or 5 mg nightly, used alone or alongside fluoxetine for eight weeks, was associated with significant reductions in Fibromyalgia Impact Questionnaire scores (PMID 21158908). A systematic review pooling four studies covering 98 patients found that all of them reported positive effects of melatonin on fibromyalgia symptoms with no major adverse events, while also flagging significant heterogeneity and the need for better trials (PMID 31783341). Small literature, consistent direction, low risk profile.

Typical studied range. 3 mg to 5 mg at night in the fibromyalgia study above (PMID 21158908). More is not better with melatonin. Many people do better on less.

Timing. Thirty to sixty minutes before bed, in a dim room. Scrolling in a bright screen at the same time cancels most of the point.

Cautions. Next-morning grogginess is the common complaint and usually means the dose is too high. It adds to the sedation of pregabalin, gabapentin, cyclobenzaprine, benzodiazepines, and opioids. Caution with blood thinners, with immune-suppressing medication, and in seizure disorders. Not established as safe in pregnancy or nursing. It can affect blood sugar and blood pressure control in some people.

Magnesium glycinate at night

Covered in Tier 1. Many people find the evening dose does double duty for muscle discomfort and sleep onset. The fibromyalgia trial data sit with magnesium citrate (PMID 22271372); choosing glycinate is a tolerability decision, not a stronger-evidence decision, and I want you to know the difference.

If pain is the problem

Palmitoylethanolamide (PEA), usually paired with acetyl-L-carnitine

What it is. PEA is a fatty acid compound your own body produces that acts on inflammation and pain signaling pathways. Acetyl-L-carnitine is a form of carnitine involved in moving fats into mitochondria for energy, with additional effects on nerve function.

Why it may help. In a randomized controlled study, patients already on stable duloxetine and pregabalin were randomized either to continue that alone or to add PEA and acetyl-L-carnitine. Over 24 weeks, the group that added the two supplements showed additional significant improvement in the Widespread Pain Index, the revised Fibromyalgia Impact Questionnaire, and the modified Fibromyalgia Assessment Status score (PMID 37378482). This is one of the more practically useful studies in the whole supplement literature for fibromyalgia, because it tested exactly the situation most readers are in: already on medication, still symptomatic, wondering whether adding something helps.

Typical studied range. PEA 600 mg twice daily and acetyl-L-carnitine 500 mg twice daily, as an addition to existing prescriptions (PMID 37378482).

Timing. Split morning and evening, as studied.

Cautions. This was an add-on study, not a replacement study. Nothing here suggests reducing a prescription, and you should not change one without your prescriber. Acetyl-L-carnitine can be stimulating for some people, so favor earlier dosing if it disturbs your sleep. Carnitine is generally avoided in people with seizure disorders without medical guidance, and people with thyroid conditions should ask, since carnitine can interact with thyroid hormone action.

Acetyl-L-carnitine on its own

Why it may help. A double-blind multicenter trial randomized 102 patients with fibromyalgia to acetyl-L-carnitine or placebo. Both groups improved early. By week ten, the placebo group had plateaued while the carnitine group kept improving, producing statistically significant between-group differences in total myalgic score, tender point count, depression, musculoskeletal pain, and most quality of life measures (PMID 17543140).

Typical studied range. 1,500 mg daily by mouth in the later phase of that trial. Note honestly that the first two weeks of the study also included intramuscular injections, so the oral-only effect is less certain than the headline suggests (PMID 17543140).

Cautions. As above. Ask before combining with thyroid medication or if you have a seizure history.

If fatigue and post-exertion crashes are the problem

Coenzyme Q10

What it is. A compound your mitochondria use in the electron transport chain, meaning it sits directly in the machinery that turns food into usable energy. Sold as ubiquinone or as ubiquinol.

Why it may help. A randomized, double-blind, placebo-controlled trial of 20 patients with fibromyalgia found that 300 mg daily of CoQ10 for 40 days was associated with reduced Fibromyalgia Impact Questionnaire scores, with the most prominent reductions in the pain, fatigue, and morning tiredness subscales, alongside changes in oxidative stress and mitochondrial gene expression markers (PMID 23458405). A more recent randomized crossover trial testing a combination of CoQ10, tryptophan, and magnesium found something more mixed: the primary endpoint of fatigue did not improve significantly more than placebo, while pain, sleep quality, and total symptom impact did improve on the supplement, with good tolerability (PMID 40151031).

How to read that honestly. The most enthusiastic CoQ10 fibromyalgia results come from a small number of very small studies from a single research group, and the largest recent independent trial missed its own primary endpoint. CoQ10 is a reasonable thing to try if fatigue is your main complaint. It is not the breakthrough that supplement marketing makes it out to be.

Typical studied range. 300 mg daily, taken in divided doses, in the trial above (PMID 23458405).

Timing. With a fat-containing meal, since absorption depends on it. Earlier in the day is usually better, as some people find it activating.

Cautions. CoQ10 can reduce the effect of warfarin, so anyone on it needs prescriber involvement and monitoring. It may modestly lower blood pressure and blood sugar, which matters if you are treated for either. Not established in pregnancy or nursing.

Creatine monohydrate

What it is. A compound stored in muscle that recycles quick-burst energy. Not just for athletes.

Why it may help. A 16-week randomized, double-blind, placebo-controlled trial in fibromyalgia found that creatine raised muscle phosphorylcreatine content and improved lower-body and upper-body muscle strength compared with placebo, with no serious adverse events. Be clear about the limits: that same trial found no meaningful changes in pain, cognition, sleep quality, or quality of life (PMID 23554283).

So why include it. Because strength is not a vanity outcome in fibromyalgia. If graded movement is the intervention with the best evidence behind it, then something that makes the movement possible without

wrecking you afterward supports the thing that actually works. Just buy it for the right reason: strength and exercise capacity, not pain relief.

Typical studied range. Creatine monohydrate is most commonly used at about 3 to 5 grams daily for maintenance in adults.

Timing. Any time of day, with fluid. Consistency matters more than timing.

Cautions. Requires good hydration. Ask your clinician first if you have any kidney condition or take medications that affect the kidneys. Some people gain a small amount of water weight in the first weeks, which is expected and is not fat gain.

If your gut is part of the picture

Gut symptoms and fibromyalgia travel together often enough that ignoring the gut leaves results on the table. Research has found that the gut bacterial community in people with fibromyalgia differs from that of matched controls, with some of those differences correlating with symptom severity (PMID 31219947).

Probiotics and prebiotic fiber

Why they may help. In a randomized, double-blind, placebo-controlled trial in women with fibromyalgia, eight weeks of a probiotic was associated with reduced pain scores versus placebo, along with improvements from baseline in sleep quality, depression, and anxiety measures, while the prebiotic inulin arm improved sleep quality and pain scores (PMID 37224267). A separate pilot randomized controlled trial found that a multispecies probiotic improved measures of impulsivity and decision-making, which is a small but real signal on the cognitive side of fibromyalgia (PMID 30026567).

The honest counterweight. A larger randomized, double-blind, placebo-controlled trial of a multi-strain probiotic for the gastrointestinal symptoms of fibromyalgia found no benefit over placebo on its primary outcome or on any secondary outcome (PMID 34681287). Different strains, different targets, different results. Probiotics are not one product, and the honest summary is “mixed, possibly helpful, strain-specific, and not yet predictable.”

Typical studied range. In the positive trial, a probiotic providing 4×10^{10} colony forming units per day, or 10 grams per day of inulin, over eight weeks (PMID 37224267).

Timing. Consistent daily use. Follow the label for with or without food, since it depends on the strain and the formulation.

Cautions. Prebiotic fiber, including inulin, commonly increases gas and bloating at first and can be genuinely uncomfortable if you also have irritable bowel syndrome. Start low. People who are significantly immune-compromised, have a central line, or are critically ill should not start probiotics without medical guidance. If you are following a low FODMAP approach (see the Diet Guide), be aware that inulin is high FODMAP.

Tier 3: Ask your practitioner first

Everything in this tier has published fibromyalgia data behind it. Every one of them also has an interaction profile serious enough that I do not want you starting it from a guide, including this one. These are worth discussing. They are not worth improvising.

5-HTP (5-hydroxytryptophan)

What it is. A direct precursor to serotonin, one metabolic step closer than tryptophan.

What the research shows. A double-blind, placebo-controlled study of 50 patients with primary fibromyalgia reported significant improvement across the clinical parameters studied, with mild and transient side effects, and the authors themselves called for further controlled studies to define its value (PMID 2193835). That study is from 1990, it is small, and the literature has not meaningfully built on it since.

Why it is in this tier and not the last one. Because a serotonin precursor added to a serotonin-active prescription is the textbook setup for serotonin syndrome, which can escalate from tremor and agitation to a life-threatening emergency (PMID 37309284). Duloxetine, milnacipran, SSRIs, tricyclics, tramadol, and triptans are all common in fibromyalgia care. This is not a theoretical concern.

Dose. Deliberately not listed here. If you and your prescriber decide this is worth trying, they should set the dose, know your full medication list, and tell you exactly which symptoms mean “stop and call.”

Cautions. Do not combine with any serotonergic medication without prescriber supervision. Not for use in pregnancy or nursing. Stop before surgery and tell your anesthesia team, because several anesthetic and pain agents are serotonergic.

SAMe (S-adenosylmethionine)

What it is. A compound involved in methylation reactions throughout the body, studied for both mood and pain.

What the research shows. A six-week double-blind trial of 44 patients with primary fibromyalgia found that 800 mg daily of oral SAMe improved clinical disease activity, pain over the previous week, fatigue, morning stiffness, and mood compared with placebo, while tender point score and muscle strength did not differ (PMID 1925418). A later double-blind study in secondary fibromyalgia did not find the same benefit (PMID 9543578). Mixed, with the positive study being the older and better known of the two.

Typical studied range. 800 mg daily by mouth for six weeks (PMID 1925418).

Cautions. SAMe is serotonin-active. The same serotonin syndrome concern applies as with 5-HTP (PMID 37309284). It can also trigger agitation or mood elevation in people with bipolar disorder, so a personal or family history of bipolar illness is a specific reason to avoid self-starting it. Not established in pregnancy or nursing.

St John's wort

Included here so you know to avoid self-starting it, not because I am recommending it. It has no convincing fibromyalgia-specific evidence base, it is serotonin-active, and it induces liver enzymes and drug transporters in a way that reduces blood levels of a long list of medications, including oral contraceptives, some anticoagulants, immunosuppressants, and several antidepressants (PMID 31742659). If you are already taking it, tell every prescriber you see. Please do not add it on your own.

Iron, only if your labs say so

Low iron stores can cause fatigue that looks exactly like the fatigue you are already fighting, so this is worth testing rather than guessing. In a placebo-controlled trial of intravenous iron in iron-deficient patients with fibromyalgia, the primary endpoint was not met, while several secondary measures including Fibromyalgia Impact Questionnaire total score, pain inventory score, and fatigue score improved more than placebo (PMID 29149437). Note that this was intravenous iron given under medical supervision, which is not the same thing as a bottle of iron tablets.

Ask for ferritin plus a full iron panel and a complete blood count. If your iron stores are truly low, correcting that is a medical decision with a real dose and real follow-up testing. Unnecessary iron supplementation is not harmless: it commonly causes constipation and nausea, and iron overload is a genuine risk in people who do not need it. Iron is also a leading cause of poisoning deaths in young children, so if you keep it in the house, keep it locked away.

What I deliberately left out, and why

You have seen these on fibromyalgia supplement lists. Here is why they are not on mine.

- **NAD precursors (NMN, NR).** Interesting biology, no meaningful fibromyalgia trial evidence, expensive. If that changes, I will change this guide.
- **High-dose “detox” and chelation products.** Chelating agents and provoked heavy metal testing are not appropriate self-directed interventions, and marketed detox drops do not have the fibromyalgia evidence to justify what they cost or the risk of steering you away from care that does work.
- **D-ribose.** The most-cited fibromyalgia work here is a small open-label pilot with no placebo group, which is not enough to earn a recommendation.
- **Curcumin, boswellia, and similar anti-inflammatory botanicals.** They have research in other pain conditions, but fibromyalgia-specific human evidence is lacking, and curcumin products commonly interact with blood thinners. If you want to include turmeric, food is a reasonable place to do it.
- **Very large multi-ingredient stacks.** If you start eight products at once, you cannot tell what is working, you cannot trace a side effect, and you have quietly multiplied your interaction risk. The economics are not in your favor either.

Telling you what to skip is part of what you paid for.

Running a fair 12-week trial

This is how you find out what actually works for your body instead of collecting bottles.

Weeks 1 and 2, baseline. Change nothing. Score pain out of ten and sleep out of ten most days. Count flare days. Write down your current medications and supplements, doses included.

Week 3, add one thing. Choose the tier 1 item that fits your labs and your biggest symptom. Start at the low end of the studied range. Keep scoring.

Weeks 4 through 8, hold steady. Resist adding anything else. Most of these need six to eight weeks to show a real signal, and week-to-week fibromyalgia variability will fool you if you judge at day ten.

Week 9, review honestly. Compare your last two weeks of scores with your baseline two weeks. If nothing moved and nothing improved in how you feel, stop that product. Not every trial succeeds, and stopping is a result, not a failure.

Weeks 10 through 12, add the second thing. Same rules. One change, low end of the range, keep scoring.

Bring the numbers to your appointments. Twelve weeks of simple daily scores is more useful to your clinician than any description you can give from memory.

Realistic expectations

Let me be direct about what good looks like, because unrealistic expectations are how people quit three weeks before something would have helped.

What is realistic. Modest, real, cumulative gains. A point off average pain. Falling asleep faster. Two fewer flare days in a month. Getting through an afternoon without the crash. Those are wins that compound, and they are the kind of change the trials in this guide actually measured.

What is not realistic. A supplement making fibromyalgia go away. Anyone selling you that is selling you something. The studies behind even the better-supported ingredients here are mostly small, mostly short, and report improvements in scores rather than resolution of the condition.

Timeline. Most of these trials ran six to sixteen weeks. Give things a fair window before you judge, and do not judge on a flare week.

Combination effects. The most encouraging fibromyalgia supplement study in this guide tested supplements added on top of existing prescription treatment and structured care (PMID 37378482). That is the model I would point you toward: supplements as an addition to the foundation, not a substitute for it.

Cost sense. If a product has produced nothing after eight to twelve weeks at a reasonable dose, stop paying for it. That money is better spent on food you will actually cook, or on care that has evidence behind it.

When to seek professional care

Book with a clinician soon if:

- Your symptoms are getting worse rather than plateauing.
- Fatigue is severe, new, or dramatically different from your usual pattern.
- You have never had a proper diagnostic workup. Thyroid disease, anemia, sleep apnea, inflammatory arthritis, and other conditions can mimic or accompany fibromyalgia, and they are treated differently.
- You are on four or more prescription medications and are adding supplements.
- Sleep is broken despite good sleep habits, or your partner reports snoring or pauses in breathing.

Seek care urgently for any of these, because they are not fibromyalgia:

- New joint swelling, redness, or warmth, or a fever with your pain
- Unexplained weight loss
- New weakness, numbness, or loss of bladder or bowel control
- Severe headache unlike your usual pattern, or a sudden change in vision
- Chest pain or new shortness of breath
- Thoughts of harming yourself. Please call or text 988 in the United States for the Suicide and Crisis Lifeline, or go to an emergency department.

Signs to stop a supplement and call your prescriber right away: agitation, confusion, rapid heartbeat, heavy sweating, shivering, muscle twitching or rigidity, or diarrhea after starting a serotonin-active product. Those are the warning signs of serotonin syndrome (PMID 37309284). Unusual bruising or bleeding, yellowing of the skin or eyes, dark urine, a rash, or any new swelling also warrant a call.

Where to find quality versions of these

Nothing in this guide names a brand, and that is on purpose. The evidence supports ingredients, not labels. But supplement quality genuinely varies, so here is what to look for on your own.

Look for third-party testing. NSF Certified for Sport, USP Verified, or an equivalent independent seal means someone outside the company confirmed the bottle contains what the label claims, at the amount claimed, without common contaminants. This matters more in supplements than in almost any other consumer category, because the manufacturer, not a regulator, is responsible for verifying content before it goes on sale.

Check the form, not just the name. Magnesium glycinate and magnesium oxide are both “magnesium” on the front of the bottle and behave very differently. The forms this guide names in each entry are the forms studied.

Read the whole ingredient panel. Combination products for sleep, mood, or pain frequently include serotonin-active ingredients that do not appear on the front label. If you take any serotonin-active medication, this is where problems start.

If you would rather not evaluate all of that yourself, I keep a screened dispensary through Fullscript, which ships directly to you. I hold no inventory myself. Everything in it is screened against the quality standards above and organized by the categories in this guide.

Root Health dispensary: us.fullscript.com/welcome/drseay

Disclosure: Root Health earns a portion of dispensary purchases. That revenue supports the practice, and it is also why this guide was written to be complete and usable on its own. Every recommendation here is based on the published evidence cited in these pages, and nothing was included, excluded, ranked, or dosed based on what it earns. You can buy any of these anywhere, and several of the most useful items in this guide, including the sleep and movement work, cost nothing at all.

Working with Root Health

Everything in this guide is general education, deliberately written to be useful without knowing your labs. A personalized plan is a different thing: it starts with testing, your medication list, your history, and the symptoms costing you the most, and it sequences interventions instead of stacking them.

If you want that, a Root Discovery Session is the right first step:

***Book a free Root Discovery Session: [rootcauseunlocked.com/discovery?](https://rootcauseunlocked.com/discovery?utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement)
[utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement](https://rootcauseunlocked.com/discovery?utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement)***

Consultations and ongoing care are both available wherever you are located. Being outside North Carolina is not a barrier to working with me. Tell me where you are when you reach out and I will lay out exactly how we would work together from there.

***Get in touch: [rootcauseunlocked.com/contact?](https://rootcauseunlocked.com/contact?utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement)
[utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement](https://rootcauseunlocked.com/contact?utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement)***

If you would rather start on your own, the most useful next step is to start tracking. Symptom patterns are hard to see from inside them, and a written record turns “I think the magnesium helped” into something you and any clinician can actually read:

***Free symptom tracker: [rootcauseunlocked.com/tracker?](https://rootcauseunlocked.com/tracker?utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement)
[utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement](https://rootcauseunlocked.com/tracker?utm_source=guide&utm_medium=ebook&utm_campaign=fibromyalgia-supplement)***

Read the companion document, the **Fibromyalgia Diet Guide**, alongside this one. Several of the nutrients here work better inside an eating pattern that supports them, and the diet evidence is worth understanding on its own terms.

References

All references below were verified against live PubMed records on 2026-08-02, including a check of publication type for retractions. Full verification detail, including what each citation does and does not support, is documented in [Fibromyalgia-Citation-Ledger-Supplements-Diet.md](#).

1. Macfarlane GJ, Kronisch C, Dean LE, et al. EULAR revised recommendations for the management of fibromyalgia. *Ann Rheum Dis*. 2017;76(2):318-328. PMID 27377815
2. Jones EA, Asaad F, Patel N, et al. Management of Fibromyalgia: An Update. *Biomedicines*. 2024;12(6). PMID 38927473
3. Ali OME. Prevalence of Vitamin D Deficiency and Its Relationship with Clinical Outcomes in Patients with Fibromyalgia: a Systematic Review of the Literature. *SN Compr Clin Med*. 2022;4(1):38. PMID 35071984
4. Wepner F, Scheuer R, Schuetz-Wieser B, et al. Effects of vitamin D on patients with fibromyalgia syndrome: a randomized placebo-controlled trial. *Pain*. 2014;155(2):261-268. PMID 24438771
5. Qu K, Li MX, Zhou YL, et al. The efficacy of vitamin D in treatment of fibromyalgia: a meta-analysis of randomized controlled studies and systematic review. *Expert Rev Clin Pharmacol*. 2022;15(4):433-442. PMID 35596576
6. Bagis S, Karabiber M, As I, et al. Is magnesium citrate treatment effective on pain, clinical parameters and functional status in patients with fibromyalgia? *Rheumatol Int*. 2013;33(1):167-172. PMID 22271372
7. Boulis M, Boulis M, Clauw D. Magnesium and Fibromyalgia: A Literature Review. *J Prim Care Community Health*. 2021;12:21501327211038433. PMID 34392734
8. Ferreira I, Ortigoza A, Moore P. Magnesium and malic acid supplement for fibromyalgia. *Medwave*. 2019;19(4):e7633. PMID 31150373
9. Fattah MAA, Morsi S, Fattah SA, et al. High-dose Omega-3 Alters Serum Magnesium and Calcium Levels and Affects Fibromyalgia Symptoms: A Randomized, Double-blind, Placebo-Control Study. *Curr Rheumatol Rev*. 2025;21(4):446-454. PMID 39377412
10. Rondanelli M, Faliva MA, Miccono A, et al. Food pyramid for subjects with chronic pain: foods and dietary constituents as anti-inflammatory and antioxidant agents. *Nutr Res Rev*. 2018;31(1):131-151. PMID 29679994
11. Hussain SA, Al-Khalifa II, Jasim NA, Gorial FI. Adjuvant use of melatonin for treatment of fibromyalgia. *J Pineal Res*. 2011;50(3):267-271. PMID 21158908
12. Hemati K, Amini Kadijani A, Sayehmiri F, et al. Melatonin in the treatment of fibromyalgia symptoms: A systematic review. *Complement Ther Clin Pract*. 2020;38:101072. PMID 31783341
13. Salaffi F, Farah S, Sarzi-Puttini P, Di Carlo M. Palmitoylethanolamide and acetyl-L-carnitine act synergistically with duloxetine and pregabalin in fibromyalgia: results of a randomised controlled study. *Clin Exp Rheumatol*. 2023;41(6):1323-1331. PMID 37378482
14. Rossini M, Di Munno O, Valentini G, et al. Double-blind, multicenter trial comparing acetyl L-carnitine with placebo in the treatment of fibromyalgia patients. *Clin Exp Rheumatol*. 2007;25(2):182-188. PMID 17543140
15. Cordero MD, Alcocer-Gomez E, de Miguel M, et al. Can coenzyme Q10 improve clinical and molecular parameters in fibromyalgia? *Antioxid Redox Signal*. 2013;19(12):1356-1361. PMID 23458405
16. Rossello Aubach L, Fornes Roca X, Fernandez Alvarez ME. Effects of Coenzyme Q10, Tryptophan, and Magnesium Supplementation on Fatigue in Patients with Fibromyalgia: A Randomized Trial. *J Diet Suppl*. 2025;22(3):433-444. PMID 40151031

17. Alves CR, Santiago BM, Lima FR, et al. Creatine supplementation in fibromyalgia: a randomized, double-blind, placebo-controlled trial. *Arthritis Care Res (Hoboken)*. 2013;65(9):1449-1459. PMID 23554283
18. Minerbi A, Gonzalez E, Brereton NJB, et al. Altered microbiome composition in individuals with fibromyalgia. *Pain*. 2019;160(11):2589-2602. PMID 31219947
19. Aslan Cin NN, Acik M, Tertemiz OF, et al. Effect of prebiotic and probiotic supplementation on reduced pain in patients with fibromyalgia syndrome: a double-blind, placebo-controlled randomized clinical trial. *Psychol Health Med*. 2024;29(3):528-541. PMID 37224267
20. Roman P, Estevez AF, Miras A, et al. A Pilot Randomized Controlled Trial to Explore Cognitive and Emotional Effects of Probiotics in Fibromyalgia. *Sci Rep*. 2018;8(1):10965. PMID 30026567
21. Calandre EP, Hidalgo-Tallon J, Molina-Barea R, et al. The Probiotic VSL#3 Does Not Seem to Be Efficacious for the Treatment of Gastrointestinal Symptomatology of Patients with Fibromyalgia: A Randomized, Double-Blind, Placebo-Controlled Clinical Trial. *Pharmaceuticals (Basel)*. 2021;14(10):1063. PMID 34681287
22. Caruso I, Sarzi Puttini P, Cazzola M, Azzolini V. Double-blind study of 5-hydroxytryptophan versus placebo in the treatment of primary fibromyalgia syndrome. *J Int Med Res*. 1990;18(3):201-209. PMID 2193835
23. Mikkelsen N, Damkier P, Pedersen SA. Serotonin syndrome: A focused review. *Basic Clin Pharmacol Toxicol*. 2023;133(2):124-129. PMID 37309284
24. Jacobsen S, Danneskiold-Samsoe B, Andersen RB. Oral S-adenosylmethionine in primary fibromyalgia. Double-blind clinical evaluation. *Scand J Rheumatol*. 1991;20(4):294-302. PMID 1925418
25. Tavoni A, Jeracitano G, Cirigliano G. Evaluation of S-adenosylmethionine in secondary fibromyalgia: a double-blind study. *Clin Exp Rheumatol*. 1998;16(1):106-107. PMID 9543578
26. Nicolussi S, Drewe J, Butterweck V, Meyer zu Schwabedissen HE. Clinical relevance of St. John's wort drug interactions revisited. *Br J Pharmacol*. 2020;177(6):1212-1226. PMID 31742659
27. Boomershine CS, Koch TA, Morris D. A Blinded, Randomized, Placebo-Controlled Study to Investigate the Efficacy and Safety of Ferric Carboxymaltose in Iron-Deficient Patients with Fibromyalgia. *Rheumatol Ther*. 2018;5(1):271-281. PMID 29149437
28. Lundh A, Lexchin J, Mintzes B, et al. Industry sponsorship and research outcome. *Cochrane Database Syst Rev*. 2017;2(2):MR000033. PMID 28207928

Dr. Adam Seay, DC · Root Health · Garner, NC · rootcauseunlocked.com

Educational content only. Not medical advice. Not a diagnosis. Not a substitute for your clinician. Talk with your prescriber before starting, stopping, or changing any supplement or medication.